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The Importance of incorporating Risk Management and assessing for Perilous Traits during Job Recruitment Interviews, Performance Appraisals, and Post-critical Incidents.

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Abstract :

This paper presents a questionnaire designed to assess risk mitigation in workplaces by identifying dangerous traits in employees & interviewees. It addresses under researched traits that can quickly put workplaces and colleagues at risk, particularly in healthcare environments, but applicable to other sectors. The questionnaire covers traits such as compulsive lying, manipulateness, emotional emptiness, impulsivity, irresponsibility, anger, and anti-social behaviors. Scenarios are included to help recognize these traits and their impact. It is intended for use during recruitment, performance appraisals, and post-critical incident debriefings. The paper argues for incorporating these concepts into workplace procedures, using Employee Assistance Programs (EAPs) for related issues, and advancing research on dark-triad personality traits in occupational settings.

Keywords : Risk management, Traits, Recruitment, Employment, Performance appraisal, Critical Incidents, Leadership, Lying, Grandiosity, Maleficence.

Introduction:

Destructive behavioral qualities can be recognized by the degree to which employees and leaders exhibit hostile verbal and non-verbal behaviors. At a minimum, these may emerge as improper punishment, intimidation, insubordination, and unreasonable requests. Conversely, passive destructive behavioral characteristics are characterized by subtle attitudes & actions, lack of necessary execution and oversight, and erratic functioning within minimal structures. Both manifestations of destructive behaviors, particularly in leadership positions, can impact previously steady employees through burnout, heightened absenteeism, and a reduction in workplace productivity.

Numerous more detrimental effects may arise, particularly when a colleague encounters a complete tyrant in their environment. Consequently, an optimal recruiting interview, together with work performance evaluations and post-critical incident debriefings, should integrate the following criteria to acknowledge excessive risk. The evaluative criteria for an interviewee should encompass: degrees of self-direction and satisfaction of needs (including both extrinsic and intrinsic motivators) self-identity (encompassing reliable interpersonal functionality); empathy; personal values indicative of a robust ethical framework; self-regulation leading to healthy, effective work

conduct attitude, & productivity; conflict resolution abilities; genuine resilience; respect; autonomy; interpersonal skills; compliance with social & employment standards; non-coercive collegial interaction; honesty; sincerity; authenticity; fairness; perseverance; appropriate assertiveness; inclusivity; and relatability. The respondent would be regarded as responsible, reliable, mature in thought, self-confident, client-centered, effective through coping strategies that promote emotional stability, rational thinking, and stress management; self-aware; flexible; intuitive; possessing sound self-esteem; self-efficacious; competent; motivated; cognizant of risks and risk-averse; capable of prioritizing or delegating as necessary; adept at establishing clear boundaries; focused; reflective in and on action; perceptive; and adaptable. Valuing their contributions to work tasks and surroundings, embracing accountability, taking initiative, simplifying work conditions and chores as necessary, appreciating the significance of work activities, and eventually demonstrating respect is essential. To identify the most significant dangers to colleagues and workplace surroundings, it is proposed that the following questionnaire with measurable components be employed. This is seen in the questions offered below.

METHOD:

Essential inquiries (categorized) for incorporation in risk assessment and the detection of hazardous characteristics and behaviors during job recruiting interviews, performance evaluations, and post-adverse incident debriefings.

1. COMPULSIVE LYING & (CONFABULATION):

- a)** Is there inadequate verbal clarification of temporal and sequential elements that serve as a 'red flag' to the interviewer?
- b)** Are there bold responses regarding previous employment history & unfavorable outcomes (including adverse events)?
- c)** What is the respondent's response to the assertion: "lying, if undetected, bears no repercussions"?
- d)** Is there a clear 'bait and switch' environment in their explanations (life, employment or academia)?
- e)** Are segments of the narrative enticingly authentic yet contrived, while the rest substituted, fabricated, or erroneous?
- f)** Does she/he promptly downplay aspects of the narrative (exhibiting wrath) and providing no explanation for the sudden conclusion?
- g)** Is this process repeated without a satisfactory explanation?

2. COLDLY APATHETIC:

The conceptualization and internalization of distinctions between 'good and wrong' are within this category of inquiries.

- a)** Do you, as the interviewer, see an absence of empathy?
- b)** Can he/she discern nuanced distinctions between 'good and wrong'?
- c)** Does the respondent express diminished, self-serving interpretations of empathy, ranging to an utter inability to comprehend the notion of empathy?
- d)** Does he/she exhibit a dehumanizing demeanor, attitude, and/or communication style?
- e)** Does the interviewee exhibit an indifferent, detached, or uninterested response when presented with scenarios?
- f)** Do they exhibit a 'get over it' mentality in reaction to job challenges or distress?
- g)** In the context of healthcare and first responder professions, is there a prevailing 'get over it' mentality toward patient suffering, diagnosis, treatment, and ongoing care regimens?

3. GLIBNESS:

- a)** Does the interviewee inappropriately laugh at or mock serious situations or narratives presented?
- b)** Does he/she exhibit or articulate indifference towards suffering?
- c)** Does he/she exhibit a shallow or careless approach towards ethical responsibilities and task execution?
- d)** What is the level of sincerity of the respondent? Does she/he appear to exhibit stereotypical answers that are superficial and lack substance? -Is there evident self-satisfaction with their responses notwithstanding stated or unarticulated criticism from the interviewer(s)?
- e)** Does she/he diminish inquiries by offering polished, superficial replies?

4. INABILITY TO EXPERIENCE REMORSE:

- a)** Is there a failure to learn from mistakes, accept responsibility for actions, or offer plausible justifications for evading previous misdeeds in which the individual was directly involved, witnessed, or participated as part of a team?

b) Is there indication that the interviewee cannot recognize and accept mistakes?

c) Is there a precedent of insensitivity and inaction concerning accountability?

d) Does the respondent demonstrate difficulties in receiving negative feedback and subsequently utilizing it for alternative actions?

e) Is there an emphasis on self-centered regret statements as opposed to genuine remorse?

f) Does he/she appear apathetic to the suffering of others? Additionally, do they have deficits in 'imagining others' (the ability to empathize with another's perspective)

5. EXTENSION OF MALEFICENCE/HARM:

a) Does the respondent express a lack of understanding regarding the notion of 'do no harm'?

b) Is there evidence of visible or vocal expressions of indifference, such as 'shrugging of the shoulders,' and/or avoidance regarding the significance of risk mitigation?

c) Does a 'blame the victim' mentality exist (for example, 'too sorry, too bad'; 'losers lose'; 'she/he brought it upon themselves')?

d) As the conversation progresses, does a cynical disposition become evident in their approach to job and/or identity?

e) Is 'for the good of others' devoid of significance or nonexistent?

6. EMOTIONAL EMPTINESS (+ SCENARIO):

The utilization of a scenario (one is supplied below) is very advantageous for eliciting an evaluable answer in this category.

Post-presentation of the scenario questions:

a) Does the respondent acknowledge and answer to the following:

- patient in distress?
- Anxiety among work colleagues?
- numerous patient fatalities?
- unreported and unrecorded instances of patient fatalities?

Scenario (for emotional emptiness identification):

Interviewer to read aloud: An 83 year old female patient is being treated for a chronic pain condition.

The healthcare unit, with 60 patients, is inadequately manned. The personnel perceive a sense of urgency. Seven abrupt and unforeseen patient fatalities have occurred in the healthcare unit over the last three weeks. None of which have been recorded or documented. Staff members are apprehensive that the healthcare unit may face closure or substantial financial cuts.

b) Post-presentation of scenario, interviewer says:

- "Envision yourself as a member of this healthcare unit."
- "What specific factors of significance do you consider? "
- "What is your initial response to this situation? "
- "Consult your notes, if necessary."

7. GRANDIOSITY:

a) Does she/he display a sense of superiority towards the interviewer, certain colleagues, or former associates?

b) Are they uncooperative or seeking dominance?

c) Are their attitudes of superiority explicitly articulated or expressed non-verbally during the interview process?

d) Is there a perspective characterized by 'this is me' or 'accept it or reject it'?

e) Are there declarations that strongly suggest there are no restrictions to their achievements and that they currently possess or will soon attain rank in the top echelons?
-And that this can be presumed without effort?

f) Does the interviewer feel that he/she assumes the interview procedure is exclusively conducted for them (with no other applicants under consideration)?

8. Complete Lack Of Acceptance Of Responsibility For Own Actions + Scenario:

It is strongly advised that a scenario be provided to the applicant, pertinent to the prospective or existing job responsibilities. This incident exemplifies clinical expertise in patient care. It especially evaluates attitudes toward risk minimization, responsibility, and accountability, as well as the ability to communicate effectively, engage in reflexive cognition, and demonstrate mental acuity.

Scenario: complete lack of responsibility for own actions.

The interviewer states: "Kindly furnish one-word responses, representing your immediate reaction, to the following:"

Varying levels in clinical excellence of patient clinical care which is decided upon via the following patient factors:

- Age
- Gender
- Cultural background
- Religion
- Language barrier
- Bed limits
- Key Performance Indicators (KPIs), and DRGs (Diagnostic Related Groups)
- Insurance type
- Co-morbidities
- Value to society
- Likeability
- Pre-care history
- Known or not-known to the healthcare facility
- Treating general practitioner
- Degree of family involvement
- Clarification (where necessary) regarding interviewee responses of concern can be undertaken at the end of the scenario.

9. MANIPULATIVE/INSIDIOUS OPPORTUNISM:

- a)** Does the respondent convey memorized explanations of risk management procedures, ethical issues, and quality of work methodologies?
- b)** Is the 'rote learning' seemingly externalized, with a limited likelihood of being internalized?
-Is there evident confabulation concerning dedication to the workplace Safety and risk mitigation?
- c)** Does the respondent engage with a dismissive attitude, yet with confidence in their potential for successful recruitment or assessment completion?
- d)** Do they misuse the notion that 'everyone makes errors' when evaluating significant unfavorable events?
- e)** Is self-serving incentivization regarded as the standard?

10. IMPULSIVITY, IRRESPONSIBILITY:

- a)** Is he/she exhausted, which may lead to client de-prioritization?
- b)** Does the respondent exhibit a self-defeating disposition, distinct from genuine humility?
-For instance, do they engage in self-sabotage during interviews, particularly noticeable when The interview is progressing favorably.
- c)** Is the interviewer frequently interrupted by the respondent?
- d)** Do they spontaneously articulate responses—apparently without contemplation, correction, or remorse?

11. Unrelenting Need For Stimulation/Easily Bored:

- a)** Does the respondent exhibit negative or reactive behavioral traits?
-consistently throughout individual inquiries? What types, if any?
- b)** Is hyperactivity persistent over time?
-including throughout the interview process?
-Does she/he suggest that this is a regular, accepted, or anticipated kind of Conduct?
- c)** Are they hypersensitive to loud sounds and sirens?
- d)** Is there further information that demonstrates inadequate concentration and focus?
- e)** Is the interviewee anxious, excessively alert, or markedly disturbed during the interview?
- f)** What is their comprehension and characterization of 'burnout'?

12. Anger, Irritability, Poor Concentration, Drug Affected:

- a)** Does the interviewee exhibit signs of irritability during the interview process?
- b)** Do they exhibit anger, for instance, when asked for clarification of facts or when prompted during an interview?
- c)** Is he/she apparently influenced by a stimulant or depressant substance?
-Are they artificially enhanced, excessively loquacious, and/or significantly distractible?
-Is there slurring of speech, excessive tiredness without a valid explanation, and/or are inexplicably confused?
- d)** Are they exhibiting agitation and hypersensitivity?

e) Is their confidence interpretable as 'jarring', 'aggressive', 'offensive', and/or conveyed through 'rudeness'?
-Has this transpired on multiple occasions, excluding any elucidation?

f) If so, are the irate reactions equivalent to acrimony?
-Is he/she hostile towards diverse work settings or worker classifications?
-The utilization of anger as a means of coercive control or intimidation to achieve personal gain advantage?

13. POOR BEHAVIORAL CONDUCT:

a) Does the interviewee employ diversionary methods, such as shifting to irrelevant topics or ideas?

b) Does he/she behave in an overly familiar or unprofessionally intimate manner with the interviewer(s)?

c) Does the respondent exhibit mood lability or emotional/psychological erraticism when answering questions?

Examining the interviewee's background becomes essential once recognizing inadequate behavioral conduct.

Interviewer's guide:

It is essential to thoroughly evaluate and consider oral and micro-gestural cues. This pertains to the potential presence of Acute Stress Disorder (ASD), Post-Traumatic Stress Disorder (PTSD), Complex Post-Traumatic Stress Disorder (CPTSD), emotional weariness, and burnout. This can be illustrated by various symptoms observed during the interview: compulsive mention of a traumatic workplace event; dissociation (re-experiencing trauma); notable memory deficits; and involuntary tremors when recounting a previous encounter. The utilization of psychological assessment methods is strongly advised when issues are apparent.

14. Anti-Social/ Exploitative Lifestyle Behaviors;

Self-disclosure is promoted, although coercion is eschewed. This entails gathering data and conducting an evaluation of antisocial or dishonest behavior. A structured questionnaire evaluation, such as personality assessments, is highly recommended.

a) What is the respondent's reaction to the subject of substance misuse prior to work (within 12 to 24 hours before shifts)?

b) Has the respondent revealed any actual or potential threats of harm to others?

c) Has the respondent revealed any actual or potential dangers of self-harm?

d) Does the disclosed material indicate a tendency for 'trouble-making,' 'team division,' or the creation of workplace conflict?
-Does he/she recognize this and offer satisfactory justifications for discussion? this?. Has the petitioner provided necessary clarifications?. Alternatively, do they rationalize their vulnerability by conceding to 'okay, Conceptualizations of the 'others do it too' variety?

15. LACK OF REALISTIC LONG-TERM GOALS:

a) Has the respondent demonstrated a lack of dedication to their employment, responsibilities, position, career trajectory?

b) Is there a record of disciplinary actions?

c) Is the applicant regarded as 'suggestible' (i.e., readily swayed in their opinions or work aims, or prone to fragmentation concerning their goals) during the interview process?

d) Has he/she demonstrated attitudinal characteristics indicative of a mindset of 'I will accomplish what is necessary, for the present, and nothing more'?

e) Has the respondent exhibited behavior indicative of being coerced into participating in the interview?

f) What could be the reasons for this?

g) Is there evasion concerning their future in the workplace?

16. HISTORY OF CHILDHOOD AND ADOLESCENT BEHAVIORAL PROBLEMS:

The interviewer is urged to obtain self-disclosure from the applicant regarding their previous challenges. The provision of the cue 'everyone has engaged in mischief in their past' (and related phrases) is advantageous.

a) Encourage the respondent to elucidate the distinctions between their adult habits and past experiences from childhood and adolescence.

b) Does the responder exhibit evident dismissiveness towards their history or resistance, such as asserting that there is 'nothing wrong' with criminal activities throughout adolescence or childhood, even if they persist into adult offending?

17. CRIMINAL ADAPTABILITY:

a) Does he/she possess a history of convictions (including remand, reprimands, or charges) encompassing civil offenses, Apprehended Violence Orders (AVOs)/Restraining Orders, or other pertinent offenses that have transpired from maturity to the present?

b) Do they provide 'negative' responses (e.g., denial of a history of crimes/offenses) while the interviewer is fully cognizant that the crimes/offenses are documented and indeed exist?

-If so, what types of offenses have been alleged or perpetrated?

c) What risk mitigation strategies are in place concerning threats to a colleague's physical safety?. Psychological safety?

d) What potential implications does the aforementioned information have on colleagues and the workplace?

-Responsibilities and workplace conditions?

Note to Interviewer/s:

Please Note: In the event of 'red flags' or warnings during the interview, conventional protocol applies. Structured questionnaires, commonly employed for psychological or other assessments, along with various interview forms, are essential for facilitating decision-making in recruiting, performance appraisal outcomes, and post-adverse events, and are strongly advised. Prior to issuing a recruitment suggestion; successful performance evaluation result; advancement; behavior report/ Recommendations following a significant unfavorable event (and so forth). The aforementioned set of inquiries, classifications, & associated information is intended solely as a preliminary reference.

Discussion & Results:

Referred to as 'corporate psychopathy', it is frequently fostered or even perpetuated by management and leaders across many work environments. This phenomena is termed corporate psychopathy, as it posits that the corporate milieu preferentially selects for psychopathic traits, including but not limited to executive leadership. The notion of 'corporate psychopathy' & its related characteristics can be fully applied to 'workplace psychopathy' (i.e., non-leadership positions). It can undoubtedly be executed by any employee, irrespective of their level, across various work situations. Many organizations, facilities, and collaboratives actively seek recruitment based on comparable attributes of concern. To clearly identify the detrimental impacts of such an individual, the following description is provided. The allure of particular work environments for corporate psychopaths is fundamentally rooted in power, status, accolades, and compensation. These encompass corporate environments that promote & incentivize competitiveness, offer substantial financial benefits, prioritize immediacy over long-term planning, and maintain optimistic expectations of employee charisma. The perilous dysfunction's capacity to thrive on chaos, remain composed during workplace challenges, employ continuous strategic thinking and behaviors, exhibit creativity, and maintain unwavering confidence is frequently esteemed by higher management rather than by their immediate colleagues.

Excellent communication abilities are regarded as a desirable attribute when concomitant with the aforementioned attributes. If strikingly descriptive, the 'on face value' working style is esteemed (by the firm) more emphatically than the aspects of productivity, excellent leadership abilities, and satisfactory performance within teams. Consequently, the very troubling underperformer is more readily elevated to a superior role. The experiences of a corporate psychopath's co-workers generally diverge significantly from those of upper management. The colleague, regardless of their hierarchical position or managerial relationship, observes & endures the consequences of the dysfunctional individual's risk-taking, unethical decision-making, bullying methods, and threats. One must analyze the various stages and aspects pertaining to the integration of an anti-social individual or psychopath into a corporate environment. The recruitment phase necessitates the recruiter's insight. Otherwise, the recruiter will succumb to the subsequent strategic tactics of the corporate psychopath: eloquence; charisma; smoothness; unwavering confidence; remarkable persuasive abilities; highly engaging, complimentary, and humorous demeanor; flawless mannerisms and gestures; & an overall impression so remarkable that it is difficult to believe the individual before you is genuine—yet they are ultimately perceived as ideal for the role. A rapid career advancement subsequently follows. Specifically, this entails the absence of scruples; ruthless manipulation; complete disloyalty to colleagues; a lack of fairness regarding seniority or promotion eligibility; pleasure derived from the demotion, termination, or suspension of others; relentless socializing; charming & deceitful behavior to ascend the hierarchy swiftly; and the strategic befriending and ingratiation within the upper echelons by any means possible. Emotional or other forms of coercion to compel colleagues to do duties on their behalf; appropriation of ideas; & imitation of top performers—stemming from a limited capacity for innovation and original output—transpire rapidly after the orientation phase of employment. Division among colleagues & teams; removal of competitive peers; attainment of elevated status & the semblance of enhanced productivity; establishment of a toxic workplace culture; & the employment of tactics that foster an environment where co-workers (rather than the culpable) appear to perpetually falter, culminate in the development of more insidious strategies. For example, character assassination (via rumor, innuendo, and blatant falsehoods); gaslighting; and violence (in multiple forms).

Conclusion:

The simultaneous use of comprehensive structured recruitment questions is strongly advised to enhance the questionnaire presented in the methods section. These may further investigate the respondent's intrinsic motivators

related to work (self-determined and independent, pursued for inherent satisfaction) and their extrinsic motivators (external variables such as dependence on approval, rewards, and monetary incentives). The implementation of the proposed or analogous questionnaire is important for inclusion in the employment interview criteria of each occupational resource department. Optimal inclusion in employment recruitment procedures and protocols. The utilization of Employment Assistance Programs (EAP) is crucial, particularly for identifying trauma issues in successful candidates or current employees. Subsequent investigations into hazardous characteristics in recruitment and among current employees may encompass the analysis of the applicability of the dark triad personality traits—Machiavellianism, narcissism, and psychopathy—alongside reduced agreeableness & heightened neuroticism, deviating from the norm. With more than 20 years of expertise in psychological intervention and health education, the author, Mary Catherine McKenzie, has devoted her life to enhancing persons' mental welfare. The author has been inspired by the grit and resilience of individuals from many backgrounds facing extensive work-related challenges, career difficulties, burnout, complicated trauma, anxiety and depression disorders, chronic pain, and grief and loss. Her enduring dedication to advocating for best practices, the advantages of red flag alerts, authenticity, and the principled application of ethics across all workplaces, particularly in healthcare, has motivated her to compose this research article.

Conflict of Interest:

The author reports no conflict of interest regarding this article.

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